The American Health Funding Premium: Why the US Pays Double for Less, and the Case for an Efficiency-First Health Lane

TAXLANE Research — panel-reviewed (revised, round 2). An educational reform model. Figures are FY2025 unless noted and are sourced to `docs/sources/source-version-ledger.md`. Rate recommendations are reform proposals; efficiency figures are conditional ceilings, not bankable savings; the borrowed share stays visible; no claim of legal dedication or tax advice is made.

Abstract

The United States funds health more heavily than any peer democracy, covers fewer people, and gets worse outcomes. Government and compulsory health spending is about **14.3% of GDP, roughly twice the OECD average of 7.1%** [`SRC-OECD-HEALTH-2025`], yet **8.0% of Americans (~27 million) were uninsured in 2024** — the US is the only high-income country without universal coverage [`SRC-CENSUS-P60-288`, `SRC-COMMONWEALTH-USHC`] — and US life expectancy (78.4 years) runs **2.7 years below the OECD average** [`SRC-OECD-HEALTH-2025`]. In the federal budget the Health and Medicare lanes cost **$1,975,229M in FY2025 — 28.2% of all outlays** [`SRC-OMB-HIST-3-2-FY2027`]. The premium is a **price** phenomenon, not higher utilization [`SRC-JAMA-PAPANICOLAS-2018`]. We argue the evidence does *not* support raising the health-funding rate; it supports an **efficiency-first lane** that bends cost-per-outcome toward peer levels under a binding coverage floor. Moderate efficiency scenarios (10/20/30%) define **conditional ceilings of up to ~$198B / ~$395B / ~$593B** — gated by verified, coverage-protected unit-price reductions, and *not* achievable by the funding lever alone (delivery reform is out of scope). A named lane improves accountability and legibility, not spending control by itself (fungibility limits any earmark). The modernization case is to fund coverage in full while making the price of care legible and accountable, rather than borrowing for an open-ended unit-cost premium that the aging population will compound.

1. Introduction

Most budget debates ask whether to spend more or less on health. That framing misses the American anomaly: the US already spends about twice what peers spend, covers fewer people, and gets worse outcomes. The question is therefore not "more or less," but **"why so much, for so little — and how should the funding be modernized?"**

Our contribution: (i) we locate the health lanes precisely in the FY2025 budget and their financing; (ii) we benchmark US health funding against peers on a scope-matched basis, on spending *and* outcomes; (iii) we show that the fair-rate factors converge on an efficiency-first rate path; and (iv) we analyze the long-run (aging) trajectory and the distributional incidence, and specify the lane and operating rule that make the path accountable.

2. What the US funds today

Lane	FY2025 cost ($M)	Share of outlays	Financed by
Health (non-Medicare)	978,511	14.0%	general revenue
Medicare	996,718	14.2%	HI payroll (2.9%) + premiums + general
Total	**1,975,229**	**28.2%**	mixed

Source: `SRC-OMB-HIST-3-2-FY2027`; financing from `SRC-OMB-HIST-2-4-FY2027`. Only Medicare Part A is payroll-funded (HI receipts $395,350M); the majority of Medicare (Parts B/D) and all of the non-Medicare lane are **general revenue**. (Note: the HI receipt figure $395,350M is distinct from the 20%-efficiency figure $395,046M in §6 — different quantities that happen to be numerically close.) Health is thus overwhelmingly a general-fund commitment, not a self-funded insurance lane — a fact obscured by debate that treats "Medicare" as fully pre-paid.

3. History: how health came to dominate the budget

Health's budget weight is recent. In the modeled allocation of federal outlays by decade, the broad **human-resources** category rose from **16.5% of the dollar in the 1940s to 69.3% in the 2020s**, while national defense fell from 60.9% to 12.4% (derived record: `income_tax_outlay_model` decade summary, ledger-registered). The growth is concentrated in a lane whose **unit cost**, not only its caseload, runs far above peers — though, as §7 shows, demographic caseload growth is the dominant *forward* driver.

4. Peer benchmark: spending and outcomes

On the OECD scope-matched measure of government/compulsory health spending:

Country	Gov/compulsory health, % GDP (2024)
United States	**14.3**
Germany	10.6
France	9.7
Sweden	9.7
United Kingdom	9.1
Japan	9.0
Netherlands	8.3
Canada	7.9
OECD average	**7.1**

Source: `SRC-OECD-HEALTH-2025`. **Scope (binding):** the OECD "government/compulsory" measure counts US compulsory *private* insurance, so the **system-level 14.3%** is broader than the **federal lanes (14.0% + 14.2% of the federal dollar)** in §2 — two different denominators (GDP vs the federal outlay dollar) that must not be conflated. US total (public+private) health spending is ~17.2% of GDP.

The premium does not buy better results. The US covers fewer people (8.0% uninsured; only high-income country without universal coverage) and posts **life expectancy 78.4 years, 2.7 below the OECD average**, with **preventable mortality 217 vs 145** and **treatable mortality 95 vs 77 per 100,000** [`SRC-OECD-HEALTH-2025`, `SRC-CENSUS-P60-288`, `SRC-COMMONWEALTH-USHC`]. So the cost-per-*outcome* gap is larger than the cost-per-GDP gap.

5. Why it costs more: price, not volume

The cross-country evidence is that higher US spending is driven chiefly by **prices** (provider and pharmaceutical prices, labor, administrative cost), not by higher utilization: US use of care is broadly similar to peers, while prices are far higher [`SRC-JAMA-PAPANICOLAS-2018`, Papanicolas et al., JAMA 2018].

This is the load-bearing fact for the rate recommendation: a price-driven premium is addressed by bending unit price, not by cutting coverage or raising the funding rate.

6. All factors → the recommended rate

Decomposing the recommendation:

- **Solvency.** Health + Medicare is 28.2% of a federal dollar that is 25.3% borrowed [`SRC-OMB-HIST-1-1-FY2027`]; a rate increase at current unit cost deepens the gap.
- **Historical.** The lane grew on unit cost, so the signal points at price.
- **International.** Decisive: ~2x peer cost for narrower coverage and worse outcomes — reduce cost-per-outcome, do not raise the rate.

The three converge on **efficiency-first**: hold or reduce the rate while bending unit cost. Costed scenarios on the federal lanes, stated as **conditional ceilings**:

Efficiency gain (cost-per-outcome)	Ceiling freed ($B)
10%	~198
20%	~395
30%	~593

Source: `health_efficiency_scenarios` (ledger-registered). These are **upper bounds conditional on a verified, coverage-protected unit-price reduction** — not bankable savings and not achievable by the funding lever alone; the delivery mechanism is out of scope (§10). They are also a **one-time level shift**, not a bend in the growth *rate* (§7), so against a rising cost curve they erode unless the trajectory also changes. We therefore do not claim a fixed share of the deficit is "closed."

7. The long-run driver: aging and the cost path

Health is the federal budget's principal long-run cost driver, and the dominant forward mechanism is **demographic**, not only unit price. CBO projects net Medicare spending rising from **3.1% of GDP (2025) to 5.2% (2055)** and the major federal health programs from **5.8% to 8.1% of GDP**, as the 65+ population grows; outlays for Social Security, Medicare, and Medicaid for people 65+ rise from **40% to over 50% of federal noninterest spending** [`SRC-CBO-LTBO`]. A unit-cost efficiency gain (§6) is a level shift against this rising path; durable solvency requires bending the growth rate, of which price is one lever and demographic/benefit design is another.

8. Distribution: who pays the premium and who gains

A funding recommendation must say who bears it. The premium's financing is **regressive-to-mixed**: the **HI payroll tax (2.9%, uncapped on wages)** is proportional to wages but regressive against total income (it exempts capital income); **general revenue** (chiefly the progressive individual income tax) is more progressive; and the **compulsory private** share folded into the 14.3% falls heavily on lower- and middle-income insured households as a share of earnings. An efficiency squeeze is **not distributionally neutral**: a unit-price reduction is income to patients and taxpayers but lost revenue to providers — and some of that revenue sustains safety-net, rural, and Medicaid-dependent systems, so the coverage floor must protect the **income gradient of access**, not only the enrollment count. Beneficiary incidence differs sharply: **Medicare seniors** (fixed income, premium-paying) and **Medicaid**

enrollees (low income, zero premium) face opposite effects from any reform and should not be collapsed into one aggregate. A full decile-incidence estimate is deferred (it needs the distributional microdata), but the direction is clear enough to require that efficiency gains be captured from high-price actors, not from providers serving low-income populations.

9. The needs, the problem, and the modernization case

Why the US struggles here. The premium is a *price* phenomenon — higher prices per service, administrative fragmentation across many payers, weak price transparency — compounded by an aging caseload. Funding the lane at this unit cost means taxpayers buy less health, for fewer people, than any peer population.

The modernization case. Modernizing health *funding* (distinct from delivery reform) means making the lane's **price legible and accountable**: publish the cost-per-outcome the lane buys; tie any efficiency saving to a verified, coverage-protected reduction in unit price; and stop financing open-ended unit-cost and demographic growth through borrowing. Fund coverage in full; refuse to borrow for a price premium that delivers worse outcomes.

10. Reform design: the efficiency-first health lane

The Health and Medicare lanes carry the standard lane object-model fields, with these health-specific rules:

- `allocation_method = proposed_reform`; general-revenue funded with the Medicare-HI payroll piece shown separately.
- **Coverage floor (binding).** A "down on efficiency" change requires a coverage/enrollment floor **and** independent verification that outcomes and the served population held constant *before* any saving is booked; a cut indistinguishable from reduced service does not qualify. The verifier is itself a political object that can be gamed (relabeling cuts as efficiency), so the floor must be measured against published, independent outcome metrics.
- **No rate increase to fund unit-cost growth** without a published cost-per-outcome justification; deficit/borrowed-share context required on every health-lane view.

11. Discussion and limitations

- **Scope.** The 14.3%-vs-7.1% figure is system-level (includes US compulsory private insurance); the federal lanes are the public slice. We do not claim the federal lanes alone can be halved to OECD levels.
- **Efficiency is contested.** A 20% cost-per-outcome reduction with coverage intact is the most-attempted, most-failed maneuver in US health policy; the scenarios are conditional ceilings gated by verification, not realized cash, and a level shift, not a growth-rate bend.
- **Fungibility.** A "Health" label does not control spending; control lives in the appropriation and coverage-floor rules.
- **Distribution.** A full decile-incidence estimate is deferred pending microdata.
- **Delivery reform** (how price is bent) is out of scope; this paper addresses the *funding* question and the accountability rule.

12. Conclusion

The American health funding premium is the clearest fair-rate finding in the federal budget: roughly double the peer cost share, for fewer people and worse outcomes, driven by price. The evidence does not support a higher health-funding rate; it supports an efficiency-first lane that bends unit cost toward peer levels under a binding coverage floor — defining conditional ceilings of up to ~$198B–$593B, gated by verified, coverage-protected price reductions and not achievable by the funding lever alone. Against an aging trajectory, durable solvency needs the growth rate bent, not only a level saving. Modernizing health funding means making the price legible and accountable — funding coverage in full while refusing to borrow for a premium that buys worse outcomes.

Sources

`SRC-OMB-HIST-3-2-FY2027`, `SRC-OMB-HIST-2-4-FY2027`, `SRC-OMB-HIST-1-1-FY2027`, `SRC-OECD-HEALTH-2025`, `SRC-CENSUS-P60-288`, `SRC-COMMONWEALTH-USHC`, `SRC-JAMA-PAPANICOLAS-2018`, `SRC-CBO-LTBO`, plus the ledger-registered derived records `income_tax_outlay_model` (decade summary) and `health_efficiency_scenarios`. All in `docs/sources/source-version-ledger.md`.